

Division of Global Health Protection (DGHP) Monitoring and Evaluation – FY 2026 Standard Partner-level Indicators

Overview

Below is a menu of DGHP M&E Partner-Level Indicators that span eight technical areas:

- (1) Infection Prevention and Control
- (2) Border Health
- (3) Community Mitigation
- (4) Emergency Operations and Response
- (5) Laboratory Diagnostics
- (6) Surveillance and Epidemiology
- (7) Vaccines
- (8) Field Epidemiology Training Programs

A comprehensive indicator menu inclusive of detailed descriptions of each indicator will be provided at time of award. DGHP partner-level

indicators should be incorporated into the recipient's Evaluation & Performance Measurement Plan (EPMP). The indicators shown below may change before award or in future years.

Partner-level Indicators

1. Infection Prevention and Control (IPC) Indicators

1.1. Cross-cutting IPC

- 1.1.1. Proportion of facilities that have an IPC focal person in place
- 1.1.2. Proportion of healthcare facilities providing essential services that implemented guideline-based IPC improvements
- 1.1.3. Number of healthcare facilities participating in CDC-supported healthcare detection and response networks

2. Border Health (BH) Indicators

2.1. Point of Entry (POE) General Capacity

- 2.1.1. Proportion of POE with multisectoral Standardized Operating Procedure (SOPs) in place for identification (signs and symptoms), notification, and illness response, including for COVID-19, among travelers
- 2.1.2. Among those with SOPs above: Proportion of POE with health- and non-health staff trained on SOPs for identification (signs and symptoms), notification, and illness response, including for COVID-19, among travelers in the last year
- 2.1.3. Number of POE that can demonstrate capacity for coordinated response in identification, notification, assessment, and referral of an ill traveler to health care through response to simulated or real-life events in the last year
- 2.1.4. Number of POE that have conducted AARs following a simulated or real-life event and implemented corrective actions to address gaps identified

- 2.1.5. Proportion of non-health POE personnel who have been trained on approved procedures and guidance in enforcing public health regulations applicable to points of entry, travel, and the transport of goods across international borders
- 2.1.6. Proportion of POE where staff have been trained on approved procedures in enforcing public health regulations applicable to points of entry, travel, and the transport of goods across international borders
- 2.1.7. Proportion of POE that can demonstrate capacity for identification, notification, assessment, and referral of an ill traveler to health care through response to simulated or real-life events in the last year
- 2.1.8. Proportion of POE health staff trained on SOPs for identification (signs and symptoms), notification, and illness response, including for COVID-19, among travelers in the last year
- 2.1.9. Proportion of POE non-health staff trained on SOPs for identification (signs and symptoms), notification, and illness response, including for COVID-19, among travelers in the last year

2.2. POE Infrastructure

- 2.2.1. Proportion of POE that have identified areas to isolate ill travelers for assessment and while waiting for transfer to a healthcare facility
- 2.2.2. Proportion of POE with sufficient equipment (e.g., Personal Protective Equipment (PPE), thermometers, forms, job-aids, handwashing stations, decontamination and disinfection supplies) or the supply chain to receive sufficient equipment to identify, notify, and respond to communicable disease illness among travelers for one month

2.3. POE IPC

- 2.3.1. Proportion of POE that implement personal protective measures (e.g., handwashing, wearing face coverings, social distancing) for staff and travelers according to developed SOPs

2.4. POE Risk Communication

- 2.4.1. Proportion of POE disseminating risk communication materials tailored for travelers in appropriate languages
- 2.4.2. Proportion of POE with staff trained on providing risk communication to travelers within the last year
- 2.4.3. Proportion of POE staff trained on providing risk communication to travelers within the last year

2.5. Data and Surveillance Systems for Mobile Populations

- 2.5.1. Number of staff trained on collecting data on population mobility patterns in the last year
- 2.5.2. Number of staff trained on analyzing and summarizing data collected on population mobility patterns in the last year
- 2.5.3. Number of priority geographic areas in which data on population mobility patterns has been collected in the last year
- 2.5.4. Proportion of public health emergency responses that utilized population mobility pattern data to inform public health interventions and/or public health emergency responses (i.e., identified POE for capacity building, identified healthcare facility (HCF) for strengthened surveillance) within the last year
- 2.5.5. There are protocols and/or standard operating procedures that govern use of border health data and information systems
- 2.5.6. Proportion of POEs with border health personnel trained on the use of established border health data and information systems according to established SOPs
- 2.5.7. Proportion of POEs with border health personnel who demonstrate use of established border health data and information systems according to established SOPs

2.6. Cross-border Coordination

- 2.6.1. Proportion of neighboring countries included in formalized agreements and procedures for public health information sharing with neighboring countries
- 2.6.2. Proportion of subnational administrative levels that have shared contact information with neighboring countries within the last year
- 2.6.3. Proportion of subnational administrative levels routinely sharing public health information with neighboring countries within the last year
- 2.6.4. Number of cross-border meetings at subnational administrative level to support operationalization of agreements and procedures within the last year

2.7. BH Operational and Legal Frameworks

- 2.7.1. Support was provided to develop an operational plan that defines the roles and responsibilities of the country's border health authority
- 2.7.2. Proportion of identified border health staff roles with finalized position descriptions at national, subnational, and POE level
- 2.7.3. Does the country have an established plan for their border health personnel training program to ensure that all border health officers can competently conduct public health operations within their jurisdiction?
- 2.7.4. National authorities have developed procedures and guidance describing how public health regulations applicable to points of entry, travel, and the transport of goods across international borders will be enforced and have disseminated them to all applicable agencies, including at subnational- and point of entry-levels
- 2.7.5. Proportion of border health personnel who have been trained on approved procedures in enforcing public health regulations applicable to points of entry, travel, and the transport of goods across international borders

3. Community Mitigation Indicators

3.1. Country Operations Support

- 3.1.1. Number of risk mitigation strategies that have been implemented that are tailored to the needs of specific populations
- 3.1.2. Proportion of frontline workers that had an increase in awareness and knowledge to plan and implement Risk Communications and Community Engagement (RCCE) interventions at various levels as determined by pre- and post- test assessment

3.2. Community Mitigation

- 3.2.1. Percent of staff trained that are active in case investigation or contact tracing during reporting period

4. Emergency Operations and Response Indicators

4.1. Strengthening of International Emergency Response Capacity

- 4.1.1. Number of workshops/trainings on Emergency Management Systems Integration (EMSI), Rapid Response Team (RRT) management and responder readiness, Risk Communication and Community Engagement (RCCE), and/or Public Health Emergency Management (PHEM)
- 4.1.2. Number of participants trained in emergency management systems integration
- 4.1.3. Number of participants trained in RCCE
- 4.1.4. Number of CDC trained (PHEM) fellows currently being utilized in country
- 4.1.5. Number of policies, plans, processes, and SOPs established for EMSI, RRT, RCCE, and/or PHEM
- 4.1.6. Country has capacity for Emergency Operations Center (EOC) activation within 48 hours of detection of a public health event
- 4.1.7. Country has capacity for RRT deployment within 48 hours of detection of a public health event

- 4.1.8. A national RCCE strategy and operations plan has been established or updated and approved
- 4.1.9. A national RCCE training package has been established and approved in the last 6 months
- 4.1.10. A national RCCE training package has been implemented within 6 months of approval
- 4.1.11. Number of public health leaders, government officials, or media spokespersons trained in RCCE
- 4.1.12. Percent change in awareness and knowledge of public health leaders and community, government officials, or spokespersons to plan and implement RCCE interventions as determined by pre-and post-test assessment
- 4.1.13. Number of strategic behavior change/risk communication messages and/or products developed for target population(s)
- 4.1.14. Number of trained RRT responders who served as responders in the field or in response coordination roles that were trained/supported by the country's emergency coordination entity (e.g., National Public Health Institute (NPHI), Public Health Emergency Operations Center (PHEOC), or country equivalent) and served in this function for the COVID-19 response
- 4.1.15. Number of workforce development trainings the NPHI conducted with neighboring countries to support public health implementation efforts
- 4.1.16. Proportion of rapid response team (RRT) staff/members trained on One Health specific principles and practices
- 4.1.17. The country's PHEOC has a formal, multi-sectoral advisory group or steering committee

4.2. Strengthening of International Emergency Operations

- 4.2.1. Number of EOCs established and/or strengthened with the associated systems within national public health institutes and at subnational levels
- 4.2.2. Does the partner provide support for recovery operations planning and implementation?
- 4.2.3. COVID-19 strategic response and recovery plan has been implemented at the country level with support of the country's emergency coordination entity (e.g., NPHI, PHEOC, or country equivalent)
- 4.2.4. A national RCCE strategy and operations plan has been used and/or tested in an exercise or response with key response interest holders within the first year after being approved
- 4.2.5. Extent to which the national public health institute (NPHI) has a legal framework establishing its operations
- 4.2.6. Extent to which the NPHI has developed a strategic plan

4.3. Multilateral Emergency Response Support

- 4.3.1. Number of information sharing systems or mechanisms established within national public health institutes/Ministry of Health and among key interest holders
- 4.3.2. Number of platforms developed for integrated data sharing at the national and subnational levels
- 4.3.3. Number of activities the NPHI has implemented with neighboring countries to support public health implementation efforts
- 4.3.4. Number of joint outbreak responses implemented using a One Health approach during the reporting period

5. Laboratory Diagnostics Indicators

5.1. Laboratory Training and Technical Assistance

- 5.1.1. Number of training tools developed for testing, biosafety, laboratory quality, and biosecurity

- 5.1.2. Number of training of trainer (TOT) sessions held by training for testing, biosafety, laboratory quality, biosecurity
- 5.1.3. Number of participants trained by training area for testing, biosafety, laboratory quality, biosecurity
- 5.1.4. Number of supported sites that received non-training related technical assistance
- 5.1.5. Number of staff documented as receiving laboratory training with certificate of completion for testing, biosafety, laboratory quality, biosecurity
- 5.1.6. Number of staff certified as competent in testing, biosafety, laboratory quality, biosecurity
- 5.1.7. Number of animal health laboratory staff trained in the latest diagnostic methods (e.g., serology, molecular, Whole Genome Sequencing (WGS))

5.2. Laboratory Quality Control/Quality Assurance

- 5.2.1. Number of rounds of proficiency testing performed for supported sites conducting COVID-19 testing in any of three areas: molecular, antibody, and/or antigen
- 5.2.2. Number of supported sites participating in External Quality Assurance Programs (EQAP)
- 5.2.3. Number of supported sites with laboratory staff participating in approved Proficiency Testing (PT) programs
- 5.2.4. Number of supported sites requesting Quality Assurance / Quality Control (QA/QC) technical assistance
- 5.2.5. Number of supported sites participating in EQAP that achieved successful/passing score
- 5.2.6. Number of supported sites participating in EQAP that achieved a score of 100%

- 5.2.7. Number of supported sites that participated in EQAP and did not achieve a successful result for every sample in the panel (qualitative) or scored less than 100% (quantitative)
- 5.2.8. Number of supported sites participating in EQAP and/or approved PT programs and did not achieve a successful result that documented what corrective action would be, or was, taken to address unsuccessful results
- 5.2.9. Number of supported sites participating in EQAP that achieved less than 100% score in more than one consecutive round
- 5.2.10. Number of supported sites participating in approved PT programs that achieved a score of 100%
- 5.2.11. Number of supported sites that participated in approved PT and did not achieve a successful result for every sample in the PT panel (qualitative) or scored less than 100% (quantitative) that documented what corrective action would be, or was, taken to address unsuccessful results
- 5.2.12. Number of supported sites using protocols authorized for United States (US) Food and Drug Administration (FDA) Emergency Use Authorization (EUA)
- 5.2.13. Number of supported sites using protocols not authorized for US FDA EUA
- 5.2.14. Have you supported laboratories in the supported country to implement routine specimen referral systems and transport networks with defined and tracked turnaround time targets?
- 5.2.15. Number of data or digital systems developed or maintained
- 5.2.16. Number of laboratory staff participating in approved PT programs that achieved successful/passing score
- 5.2.17. Number of laboratory staff that participated in approved PT programs and did not achieve a successful/passing score

5.3. Laboratory Procurement

- 5.3.1. Number of last-mile (in-country) deliveries of laboratory goods for [insert order type] financially supported and completed
- 5.3.2. Number of last-mile (in-country) deliveries of laboratory goods for [insert order type] financially supported and completed under appropriate storage and transport conditions
- 5.3.3. Number of consignees that received financial and/or logistics support for procurement of laboratory reagents and/or other supplies necessary to conduct laboratory testing

6. Surveillance and Epidemiology Indicators

6.1. Data to Influenza-Like Illness / Severe Acute Respiratory Infection (ILI/SARI) Platforms

- 6.1.1. Extent to which technical assistance has been provided to country for weekly surveillance reporting on the weekly number of new confirmed cases, deaths, and hospitalizations, including in HCWs, disaggregated by age, sex, and geographic region
- 6.1.2. Country is planning or implementing seroprevalence studies

6.2. Surveillance-related Trainings

- 6.2.1. Number of trainings held on data management and epidemiologic analysis for integrated respiratory disease surveillance
- 6.2.2. Number of staff (Ministry of Health and other organizations) at local and national level dedicated to supporting and analyzing surveillance data and activities

6.3. Surveillance-related Activities

- 6.3.1. Was surveillance, laboratory, and outbreak data/information shared at least once in country during the reporting period?

- 6.3.2. Is surveillance, laboratory, and outbreak data/information shared between sectors through routine channels at regular intervals in the country?

6.4. One Health

- 6.4.1. Proportion of One Health case results reported to relevant One Health sectors
- 6.4.2. Number of people per sector attending One Health trainings

7. Vaccine Indicators

- 7.1. Program Evaluation
- 7.2. Technical assistance was provided to conduct evaluations of essential immunization programs and immunization campaigns (not specific to COVID-19 vaccines)

8. Field Epidemiology Training Programs (FETP) Indicators

8.1. Program Implementation Indicators

- 8.1.1. Number of FETP trainees and graduates accessing curriculum that is adapted to integrate emergency management competencies
- 8.1.2. National Public Health Institutes (NPHIs) are engaged in Emergency Operations Center (EOC) strengthening and systems integrated activities including FETPs
- 8.1.3. Proportion of FETP trainees at each level [frontline, intermediate, and advanced] who were trained on One Health specific principles and practices

Performance reports will be based on which Notice of Funding Opportunity (NOFO) activities and outcomes are identified in the logic

model. Performance reports will be submitted to CDC in accordance with the requirements listed under this NOFO and overall performance will be reviewed on regular technical calls as well as through joint strategic review meetings.